Materials Design Analysis Reporting (MDAR) Checklist for Authors

The MDAR framework establishes a minimum set of requirements in transparent reporting applicable to studies in the life sciences (see Statement of Task: doi:10.31222/osf.oio/9sm4x.). The MDAR checklist is a tool for authors, editors and others seeking to adopt the MDAR framework for transparent reporting in manuscripts and other outputs. Please refer to the MDAR Elaboration Document for additional context for the MDAR framework.

Materials

Antibodies	Yes (indicate where provided: page no/section/legend)	n/a
For commercial reagents, provide supplier name, catalogue number and RRID, if available	No antibodes detected. Please add identifiers for all resources where possible	

Cell Materials	Yes (indicate where provided: page no/section/legend)	n/a
Cell lines: Provide species information, strain. Provide accession number in repository OR supplier name, catalog number, clone number, OR RRID	No cell lines detected Please add identifiers for all resources where possible	
Primary cultures: Provide species, strain, sex of origin, genetic modification status.	Not currently checked by SciScore	

Experimental Animals	Yes (indicate where provided: page no/section/legend)	n/a
Laboratory animals: Provide species, strain, sex, age, genetic modification status. Provide accession number in repository OR supplier name, catalog number, clone number, OR RRID	No organisms detected Please add identifiers for all resources where possible	
Animal observed in or captured from the field: Provide species, sex and age where possible	Not currently checked by SciScore	
Model organisms: Provide Accession number in repository (where relevant) OR RRID	See laboratory animals section for information.	

Plants and microbes	Yes (indicate where provided: page no/section/legend)	n/a
Plants: provide species and strain, unique accession number if available, and source (including location for collected wild specimens)	Not currently checked by SciScore	
Microbes: provide species and strain, unique accession number if available, and source	Not currently checked by SciScore	

Human research participants	Yes (indicate where provided: page no/section/legend)	n/a
Identify authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	The study protocol was approved by the Ethics Committee of Sakurabashi Watanabe Hospital (Reference no. 17-32).	
Provide statement confirming informed consent obtained from study participants.	One of the investigators obtained informed consent from each patient before they were included in the study.	
Report on age and sex for all study participants.	**Age:**not detected. **Sex:**not detected.	

Design

Study protocol	Yes (indicate where provided: page no/section/legend)	n/a
For clinical trials, provide the trial registration number OR cite DOI in manuscript.	Not detected.	

Laboratory protocol	Yes (indicate where provided: page no/section/legend)	n/a
Provide DOI or other citation details if detailed step-by-step protocols are available.	Not detected.	

Experimental study design (statistics details)	Yes (indicate where provided: page no/section/legend)	n/a
State whether and how the following have been done, or if they were not carried out		
Sample size determination	not detected.	
Randomization	Data Reproducibility Intra- and interobserver agreements in TMAD4ch, TMAD2ch, and GLS were evaluated by measuring these parameters in 10 randomly selected cases twice by the same observer or by 2 independent observers, respectively.	
Blinding	28 Information on subsequent clinical events up to 5 years was gathered by investigators by visiting outpatient clinics or by verbal or written blinded review of the circumstances surrounding each death.	
inclusion/exclusion criteria	Patients were excluded if: (1) primary PCI for an occluded or stenosed culprit vessel was not attempted during the acute phase of MI; (2) they had an arrhythmia disturbing GLS and TMAD measurement, such as atrial fibrillation and multiple extra beats; (3) they underwent incomplete echocardiographic study or their images were not stored; and (4) they had poor image quality that rendered the images unsuitable for analysis.	

Sample definition and in-laboratory replication	Yes (indicate where provided: page no/section/legend)	n/a
State number of times the experiment was replicated in laboratory	All parameters were measured in duplicate and averaged.	
Define whether data describe technical or biological replicates	Not detected.	

Ethics	Yes (indicate where provided: page no/section/legend)	n/a
Studies involving human participants: State details of authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	The study protocol was approved by the Ethics Committee of Sakurabashi Watanabe Hospital (Reference no. 17-32).	
Studies involving experimental animals: State details of authority granting ethics approval (IRB or equivalent committee(s), provide reference number for approval.	Not detected.	
Studies involving specimen and field samples: State if relevant permits obtained, provide details of authority approving study; if none were required, explain why.	Not detected.	

Dual Use Research of Concern (DURC)	Yes (indicate where provided: page no/section/legend)	n/a
If study is subject to dual use research of concern, state the authority granting approval and reference number for the regulatory approval	Not currently checked by SciScore	

Analysis

Attrition	Yes (indicate where provided: page no/section/legend)	n/a
State if sample or data point from the analysis is excluded, and whether the criteria for exclusion were determined and specified in advance.	Of these 422 patients, 290 patients who underwent echocardiography 2 weeks after the onset of MI using an iE33 (Philips Healthcare, Andover MA, USA) were enrolled in the present study.	

Statistics	Yes (indicate where provided: page no/section/legend)	n/a
Describe statistical tests used and justify choice of tests.	Comparisons were made using 1-way analysis of variance (ANOVA) for continuous variables, and the significance of differences was calculated using Tukey's honestly significant difference (HSD) test for factor analysis.	

Data availability	Yes (indicate where provided: page no/section/legend)	n/a
State whether newly created datasets are available, including protocols for access or restriction on access.	Not detected.	
If data are publicly available, provide accession number in repository or DOI or URL.	Not detected.	
If publicly available data are reused, provide accession number in repository or DOI or URL, where possible.	Not detected.	

Code availability	Yes (indicate where provided: page no/section/legend)	n/a
For all newly generated code and software essential for replicating the main findings of the study:		
State whether the code or software is available.	Not detected.	
If code is publicly available, provide accession number in repository, or DOI or URL.	Not detected.	

Analysis

Adherence to community standards	Yes (indicate where provided: page no/section/legend)	n/a
MDAR framework recommends adoption of discipline-specific guidelines, established and endorsed through community initiatives. Journals have their own policy about requiring specific guidelines and recommendations to complement MDAR.		
State if relevant guidelines (eg., ICMJE, MIBBI, ARRIVE) have been followed, and whether a checklist (eg., CONSORT, PRISMA, ARRIVE) is provided with the manuscript.	Not currently checked by SciScore	